

IMAGES IN CLINICAL MEDICINE

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Vaginal Varix in Pregnancy



A 31-YEAR-OLD WOMAN WAS REFERRED TO THE HIGH-RISK OBSTETRICS clinic at 35 weeks 2 days' gestation for evaluation of a vaginal lesion. The patient's first pregnancy and vaginal delivery 2 years earlier had been uncomplicated. The vaginal lesion was asymptomatic and had been noted during a routine third-trimester prenatal visit. On physical examination, a nontender purple mass originating from the vaginal wall on the left side protruded into the introitus (asterisk). Transvaginal ultrasonography with color Doppler revealed a mass of vascular origin measuring 55 mm by 21 mm. A diagnosis of a vaginal varix was made. Vaginal varices — also known as vaginal varicosities — are an uncommon complication in the second and third trimesters. They develop owing to physiological changes of pregnancy, including hormonal effects, increased blood volume, and mechanical compression of pelvic veins and the inferior vena cava by the enlarged uterus. After a multidisciplinary discussion regarding the approach to delivery in the context of potentially increased bleeding risk, the patient opted for a vaginal delivery. The patient had a late-term pregnancy, and labor was induced at 41 weeks' gestation. She delivered a healthy infant without complications. At a 6-week postpartum follow-up visit, the vaginal varix had resolved.

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